MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 11 OF 28

MFT 711

Child and Adolescent Therapy

15 teaching weeks · 3 practicals · 0 worked cases · 1 figures

SEMESTER TWO

1.71 units · Prerequisite: MFT 703, 706

> “Children rarely refer themselves and rarely benefit from an adult conversation.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 711 — Child and Adolescent Therapy

MSc Marriage and Family Therapy · Semester Two · 1.71 Units · 15 teaching weeks · Prerequisite: MFT 703, 706

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	**Child and Adolescent Therapy**	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	Master Thesis	Three	1.71

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Course leader's summary. Children rarely refer themselves and rarely benefit from an adult conversation. This course teaches you to work with children as agents in a family system: how to speak to them, play with them, assess developmentally, and intervene through the adults who live with them.

How to use this book

Each week has a **developmental frame** (what is normal at this age), a **clinical frame** (what is different when it goes wrong), and a **practice frame** (what you actually do in the room). The play and activity protocols are written so you can use them next week, not after you qualify.

1. Course description

Child and adolescent work in family therapy is not a specialism bolted on to adult work; it is a distinct skill set. Children communicate through play, behaviour and the body; adolescents communicate through testing, silence and peers; and both live inside systems that can either support or undermine change.

The course covers: developmental assessment, engaging children and adolescents, family-based interventions, common presentations (anxiety, behaviour problems, school refusal, trauma, self-harm, eating difficulties), divorce and blended families, safeguarding, and the interface with schools and paediatrics.

2. Learning outcomes

#	Outcome	Assessed
LO1	Conduct a developmental and systemic assessment of a child or adolescent	Observed session
LO2	Engage a child using play, drawing and conversation appropriate to age	Skills assessment
LO3	Formulate a child's presentation relationally and developmentally	Case study
LO4	Deliver a family-based intervention for at least two common presentations	Case study
LO5	Work with schools, paediatricians and child protection systems	Report
LO6	Recognise and respond to safeguarding concerns	Exam
LO7	Manage the ethical complexities of consent, confidentiality and divided loyalty	Viva

3. Assessment

Component	Weight	Due
Child assessment observed (30 min with child + 20 min feedback to carers)	30%	Week 8

Component	Weight	Due
Family-based intervention case study (2,500 words)	40%	Week 13
Examination	30%	Week 16

4. The fifteen-week plan

Week	Topic	Practical
1	Children in context: why family therapy is child therapy	Observation of a child at play
2	Developmental assessment and red flags	Milestone interview
3	Engaging children: language, play, drawing, behaviour	Drawing interview
4	Engaging adolescents: alliance, autonomy, honesty	Adolescent interview
5	Consent, confidentiality and divided loyalty	Ethics scenario work
6	Anxiety and school refusal	Family-based exposure plan
7	Behaviour problems and parenting interventions	Parent session rehearsal
8	**Child assessment observed**	Assessed
9	Trauma and maltreatment in children	Safety and stabilisation
10	Self-harm and suicidality in adolescents	Risk assessment with a young person
11	Divorce, separation and blended families	Telling-the-children protocol
12	Eating difficulties and body image	Family-based approach outline
13	**Case study due**; working with schools and paediatric services	Report writing
14	Neurodivergence in family context	Environmental adaptation
15	Endings with children; integration	Termination play session

Week 1 — Children in context

The paradox of child therapy. A child is referred because of behaviour. The behaviour occurs in a context. If you treat the child alone, you have removed them from the context that maintains the behaviour and asked them to change it on their own. Family-based work is therefore not an ideological preference; it is a practical inference.

Children as agents. Children are not passive recipients of family dynamics. They notice, interpret, protect, sacrifice and resist. A nine-year-old who refuses school may be keeping a depressed mother company; a thirteen-year-old who fights may be absorbing a father's rage to protect a younger sibling.

The evidence. Family-based treatments have strong evidence for: conduct problems (parent management training, functional family therapy, multisystemic therapy),

adolescent substance misuse, eating disorders (family-based treatment for anorexia nervosa is first-line), anxiety in children (parent-involved CBT), and psychosis relapse prevention (family intervention).

> **PRACTICAL**
> Observe a child (with consent) for 15 minutes in free play. Record only behaviour: what they did, in what order, what they returned to. No interpretation for the first ten minutes.

Week 2 — Developmental assessment and red flags



Figure 2.1 — Developmental milestones from 0 to 18

The four-category check (from MFT 703): normal variation, delay, regression, symptom.

Red flags requiring referral, not therapy alone.

Domain	Red flag
Language	Loss of words; no words at 24 months with no comprehension
Motor	Loss of motor skills; persistent asymmetry; regression
Social	Loss of eye contact and social interest; loss of pretend play

Domain	Red flag
Behaviour	Sudden change in personality; new aggression after a specific event
School	Marked decline in a previously competent child
Medical	Seizures, severe headaches, weight loss, unexplained bruising
Mental state	Hearing voices; fixed beliefs; severe withdrawal; self-harm
Safeguarding	Disclosure, injuries, sexualised behaviour, fear of a specific adult

Assessment with children. Use several modes: interview, drawing, play observation, parent report, teacher report, and standardised measures (SDQ, RCADS). Children's self-report and parents' report diverge systematically — collect both, report both.

Week 3 — Engaging children

Rules of engagement.

1. **Explain the frame to the child, not just the adult.** "We will meet four times. We will talk and sometimes draw. You can say 'pass' to any question."
2. **Get on their level.** Sit on the floor, use their name, match their pace.
3. **Use a medium.** Drawing, puppets, sand, figures, or a simple game. The medium is not a distraction; it is the language.
4. **Do not interrogate.** One question, then follow. "Tell me about this house" beats "who lives here?"
5. **Be honest.** Children detect adult pretence and are damaged by it. If you cannot answer, say so.

Three reliable techniques.

- **The three wishes.** "If you had three wishes, what would they be?" (One is usually for the child, one for the family, one impossible.)
- **Draw your family.** Then ask: "Who is this? What are they doing? Who is closest to whom?" Note who is omitted, who is drawn largest, who is at the edge.
- **The magic wand / miracle.** "If you woke up and the problem was gone, what would be different at breakfast?"

Avoid interpretation. "That must be your father" is a projection. Ask instead: "Tell me about this person."

> **PRACTICAL**
> 20-minute drawing interview with a peer playing an 8-year-old. Use the three techniques. Debrief: how many questions did you ask, and what did you learn without asking?

Week 4 — Engaging adolescents

What adolescents need from a therapist. Honesty, respect for autonomy, a genuine alliance with them (not with their parents), and confidentiality that is real except where safety requires otherwise.

The first five minutes. Speak to the adolescent first. Ask their version before the parents'. Explain confidentiality in their language: "What you tell me stays here, unless I think you or someone else is in danger. If that happens, I will tell you before I tell anyone else."

Techniques that work.

- **Third-person questions.** "What do teenagers your age usually say about this?" Lowers the exposure.
- **Scaling.** "On 0–10, how much does this bother you? How much does it bother your mum?"
- **Future questioning.** "What do you want your life to look like in two years? What would have to change first?"
- **Silence tolerance.** Do not fill every pause.
- **Negotiated goals.** "What would make this worth your time?"

What fails. Lectures, moralising, siding with parents, pretending to be young, and asking "why".

Week 5 — Consent, confidentiality and divided loyalty

Consent. For minors, a parent or guardian consents; the young person **assents**. Both matter. Record both. Where a competent adolescent seeks help without parental knowledge, follow your jurisdiction's law and your agency policy — and do not improvise.

Confidentiality with children.

Situation	Approach
Child discloses something minor	Keep it; help them tell the parent if they want
Child discloses self-harm	Assess risk; agree a plan; inform carers with the child's knowledge
Child discloses abuse	Follow safeguarding procedure; tell the child what will happen next
Parent asks what the child said	Give themes, not transcripts; protect the alliance
Parent demands a report for court	Refer; a treating therapist should not provide forensic opinion

Divided loyalty. The most common clinical trap: the child tells you something that would hurt the parent, or the parent asks you to "fix" the child's attitude. Name the dynamic and refuse to be recruited: "I am not going to be the one who tells your mother. I will help you do it."

Week 6 — Anxiety and school refusal

Anxiety in children. Developmentally normal fears (separation, darkness, monsters, social evaluation) become disorders when they are persistent, impairing and out of proportion.

The family-based approach.

1. **Psychoeducation.** Anxiety is a false alarm; avoidance reduces anxiety now and increases it later.
2. **Map the accommodation.** What does the family do to prevent the child's distress? (Answering questions for them, sleeping in their bed, allowing school avoidance, checking repeatedly.) Accommodation is usually maintained by love, and naming it must be done without blame.
3. **Graded exposure.** Together, build a ladder from easiest to hardest; start at the bottom; reward approach, not the absence of anxiety.
4. **Parental coaching.** Parents become coaches, not rescuers: "I know this is hard. I will be here at 3 o'clock."
5. **The school plan.** One named adult, a quiet space, a gradual return timetable, and no negotiation at the school gate.

School refusal — a differential. Anxiety, separation difficulty, bullying, learning difficulty, family crisis or a bid to protect a parent. The treatment differs for each. Assess by asking what happens on Sunday night, and what the child's absence makes possible at home.

> **PRACTICAL**
> Build an exposure ladder with a role-played parent for a 10-year-old with separation anxiety. Ten rungs, specific, graded, with a reward and a review date.

Week 7 — Behaviour problems and parenting

The evidence base. Parent management training (PMT) and its variants have the strongest evidence for child conduct problems. The mechanism is consistent: increase positive attention, give clear instructions, use consistent consequences, reduce harsh discipline, and rebuild the parent–child relationship.

The core skills (teach, model, rehearse, review).

Skill	What it looks like
Special time	10 minutes daily of child-led play, no correction
Praise	Specific, immediate, frequent ("thank you for putting your shoes away")
Instructions	Get attention, get close, one instruction, allow 5 seconds, praise compliance
Planned ignoring	For minor attention-seeking; not for dangerous behaviour

Skill	What it looks like
Consequences	Consistent, proportionate, predictable, delivered calmly
Routine	Predictable structure reduces conflict

Why parenting programmes fail. Too much content per session; the parent is depressed or exhausted; the child has an untreated condition; another adult undermines the plan; or the family's material circumstances make consistency impossible. Address these before adding more technique.

Adaptation. In households where several adults care for the child (grandmother, aunt, house help), bring them all in and agree one plan. A plan implemented differently by four adults is not a plan.

Week 8 — Observed child assessment

Thirty minutes with a child (role-played by a trained peer/actor), then twenty minutes of feedback to the carers. Assessed on: engagement, developmental appropriateness, information gathering without interrogation, risk screen, and the quality of the feedback.

Week 9 — Trauma and maltreatment

Principles (developed in MFT 714). Safety first; stabilise before processing; the caregiver's own regulation is the child's best medicine; and never force a narrative.

What trauma looks like in children.

Age	Presentation
0–2	Feeding and sleep disruption, startled response, withdrawal
3–6	Regression, repetitive play, nightmares, clinging
7–11	Aggression, school problems, somatic complaints, guilt
12–18	Risk-taking, self-harm, dissociation, substance use, precocious sexuality

Family work after maltreatment. The non-offending caregiver's belief and protection is the single strongest predictor of recovery. Work with them first. Where the perpetrator remains in the home, safety planning precedes everything.

Safeguarding. Know your reporting route, document verbatim disclosures without leading questions, and never promise a child secrecy about abuse.

Week 10 — Self-harm and suicidality in adolescents

Ask directly. "Sometimes when people feel this bad, they think about hurting themselves or ending their life. Has that happened for you?" Direct questions do not plant ideas.

Assess: frequency, method, function (relief, punishment, communication, control), intent, plan, means, protective factors, and the family's response.

Manage the family. Parents often respond with panic, punishment or secrecy. Help them respond with calm supervision and connection: remove means, increase monitoring without surveillance, and seek help immediately if risk rises.

Confidentiality. A young person's request for secrecy must yield to safety. Tell them what you will disclose and why, and involve them in how.

Week 11 — Divorce, separation and blended families

Telling the children. Both parents together, a simple message with no blame, permission to love both parents, and reassurance that the adults' decision is not the child's fault. No details about the affair, the money or the courts.

What harms children. Exposure to conflict (more than separation itself), being used as a messenger, being asked to choose, hearing a parent denigrated, and instability.

What protects them. A cooperative co-parenting relationship, structure, a relationship with both parents, and adults who can manage their own distress.

Blended families. Build the couple bond first; stepparents take authority gradually and through the biological parent; children need permission to love both households; and the "insider/outsider" split must be named and worked with.

Week 12 — Eating difficulties

Family-based treatment for anorexia nervosa (FBT, "Maudsley") is a first-line intervention for adolescents. It is agnostic about cause and externalises the illness: the family is recruited to defeat the eating disorder, not blamed for it.

Three phases. 1) Parents take charge of refeeding; 2) control is returned to the adolescent, age-appropriately; 3) identity and developmental issues are addressed.

Medical safety. Eating disorders carry the highest mortality of any mental disorder. Weight, vital signs and bloods are medical matters; do not work in isolation.

Avoid. Explaining the illness as a consequence of family dynamics; power struggles over food in session; and colluding with the idea that the young person can simply "decide to eat".

Week 13 — Schools and paediatric services

The school as a system. Teachers see the child in a different context and hold different data. Ask for specifics, not impressions: attendance, incidents, academic trajectory, peer relationships, and what strategies have been tried.

Writing a useful report for a school. One page: what you observed, what it means, three concrete recommendations, and who to contact. Avoid diagnosis-led recommendations the school cannot implement.

Paediatric liaison. Distinguish functional from organic symptoms without dismissing either. A referral that names what you want excluded is more likely to be answered (see MFT 707).

> **REPORT**
> Write a one-page school communication for a child with school refusal, including a graded return plan with named adults.

Week 14 — Neurodivergence in family context

Reframe. Autism and ADHD are not behavioural choices and not caused by parenting. But the environment determines how disabling they are.

Practical family work.

- Psychoeducation and demystification for siblings and grandparents.
- Environmental adaptation: predictability, visual schedules, sensory accommodations, transition warnings.
- Parental coaching: reduce the demand–avoidance cycle; increase specific praise; build in recovery time.
- Couple work: neurodivergence in a partner or child strains the parental alliance; address it explicitly.
- School liaison: accommodations are usually more effective than consequences.

Avoid. Behavioural contracts for skills the child does not yet have; blaming parents; and treating a diagnosis as an explanation for everything.

Week 15 — Endings with children

Children end differently. Use a session to review: what we did, what changed, what you will remember, and what you will do when you need help again. Use the medium you used (drawing, play) to close the loop. Give the child something concrete (a certificate, a drawing, a letter) and make sure the carers know when and how to return.

Glossary

Accommodation (of anxiety) — family behaviours that reduce a child's distress in the short term and maintain the anxiety. **Assent** — a minor's agreement to treatment, alongside guardian consent. **Externalising (the illness)** — separating the child from the disorder, as in FBT. **Family-based treatment (FBT)** — a manualised family approach to adolescent anorexia nervosa. **Parent management training (PMT)** — behavioural skills training for parents of children with conduct problems. **School refusal** — difficulty attending school owing to emotional distress, distinct from truancy. **Separation anxiety** — developmentally excessive distress about separation from attachment figures.

Self-assessment

Multiple choice

1. The strongest evidence for child conduct problems is for: (a) individual play therapy (b) parent management training (c) family sculpture (d) medication only

2. In anxiety work, family accommodation: (a) is helpful (b) maintains anxiety despite reducing short-term distress (c) is irrelevant (d) should be punished

3. Asking an adolescent directly about suicide: (a) increases risk (b) does not increase risk and is required (c) should be avoided (d) requires parental consent

4. FBT for anorexia: (a) blames parents (b) recruits parents to defeat the illness (c) focuses on insight only (d) is individual therapy

5. School refusal differs from truancy in that it involves: (a) deception without distress (b) emotional distress about attending (c) peer pressure (d) academic failure

6. A child's disclosure of abuse should be: (a) kept confidential (b) documented verbatim and reported (c) discussed with the alleged abuser (d) ignored if vague

7. The strongest predictor of recovery after maltreatment is: (a) the child's temperament (b) a protective, believing caregiver (c) the severity of abuse (d) therapy length

8. Neurodivergence is best addressed by: (a) behavioural contracts (b) environmental adaptation and skills-based support (c) blaming parents (d) exclusion

9. Divided loyalty refers to: (a) the therapist's loyalty to the agency (b) the child's conflict between therapist and parent (c) sibling rivalry (d) custody disputes

10. Telling children about divorce should: (a) include details of the affair (b) be done by both parents together without blame (c) be left to the courts (d) wait until they ask

Answer key

1 b · 2 b · 3 b · 4 b · 5 b · 6 b · 7 b · 8 b · 9 b · 10 b

Short answer

1. List six red flags requiring referral in child assessment.

2. Describe the three drawing/play techniques and what each reveals.
3. How would you explain confidentiality to a 14-year-old?
4. Give the five core skills of parent management training.
5. What are the three phases of FBT and what happens in each?

Essay (choose one, 2,000 words)

1. "There is no such thing as child therapy without the family." Discuss with evidence.
2. Using a case, show how a developmental formulation changed the treatment of a behaviour problem.
3. Analyse the ethical tensions in working with a 15-year-old whose parents disagree with her treatment goals.

Further reading

- Carr, A. (2019). Family Therapy: Concepts, Process and Practice (3rd ed.). Wiley.
- Kazdin, A. E. (2018). Parent Management Training. OUP.
- Lock, J., & Le Grange, D. (2013). Treatment Manual for Anorexia Nervosa: A Family-Based Approach (2nd ed.). Guilford.
- Sroufe, L. A. et al. (2005). The Development of the Person. Guilford.
- Siegel, D. J., & Bryson, T. P. (2011). The Whole-Brain Child. Delacorte.

Next: MFT 712 — Crisis Intervention Strategies